STROBE-MR reporting crosswalk

STROBE-MR item(s)	Location / reporting status
1 Title and abstract	Association-focused MR findings and secondary colocalization stated in Abstract; title identifies genes and diseases.
2–3 Background and objectives	Introduction; comparative TSHR/IGF1R susceptibility question, not treatment efficacy.
4 Study design and sources	Methods; Table 1; original vs post hoc analyses and revised reporting emphasis distinguished. No prospective registration.
5 Assumptions	Introduction, Methods and Limitations: relevance, pleiotropy limitations, ancestry transfer and tissue relevance.
6 Statistical methods	Wald/IVW, harmonization, effect reconstruction, thresholds, coloc hypotheses and priors in Methods/Supplementary Methods.
7 Assessment of assumptions	F statistics, Q, original alternative estimators, variant coverage; direct confounding/pleiotropy validation unavailable.
8 Sensitivity analyses	Frequency, windows, priors, omissions, allele audit and fixed GTEx panel. No multi-signal coloc or Steiger test.
9 Software and registration	Methods; pinned R/package versions; no prospective study registration.
10 Descriptive data	Table 1; 2,544 eligible genes; estimable counts; case/control counts. Individual demographics not available.
11 Main results	Results: TSHR/IGF1R associations and corroborating genes; Table 2 (15 comparisons), Table 3A–C, Figures 1–3 and full workbook.
12 Assumption assessment results	Instrument heterogeneity/concentration and coloc discordance described in Results; limitations retained.
13 Sensitivity results	Tables S1–S3; Figure S2; workbook allele and tissue sheets, including all 50 planned tissue comparisons.
14 Key results	Opening Discussion and Conclusions: consistent TSHR associations and limited IGF1R evidence; no expression-mediated causal proof.
15 Limitations	Two paragraphs; selection, overlap, ancestry, tissue, coverage, instruments, single-signal model and unperformed tests.
16 Interpretation	Eight Discussion paragraphs; A-subunit and orbital fibroblast evidence is external biological context. No GD/TED independence or treatment-prediction claim.
17 Generalizability	Blood exposure, ancestry and phenotype limitations; no patient-level subgroup conclusions.
18 Funding	Declarations; retained from existing manuscript, subject to author confirmation.
19 Data and sharing	Supplementary Dataset and source repositories; verification records and scripts in review package.
20 Conflicts	Declarations; retained from existing manuscript, subject to author confirmation.

This crosswalk documents what is and is not reported; it is not certification of methodological validity or an official journal form.